

Eye Fascination Induction — Script & Playbook

Kappasinian eye-fixation / eye-fascination induction. Synthesized from the Practicum Scripts workbook (101-5, item 15 — permissive baseline) and the classic paternal-rapid variant (forehead touch + limpness convincer). All wording original; delivery notes in parentheses.

Two deliveries live in this playbook. Pick by suggestibility read (§1): the **Permissive lane** (workbook baseline — slow, spacious, "don't close yet," blur imagery) for Emotional/inferred clients, and the **Paternal-Rapid lane** (message-unit overload, forehead touch on close, hands pushed off the lap) for Physical/literal clients. The physiology, checkpoints, and count-out are shared.

Delivery legend

(slow) / (rapid) — pacing; permissive lane is slow and spacious, paternal lane is fast and continuous

(voice ↓) — pitch/volume drop at phrase end; suggestion lands

(voice ↑) — brighten/lift; emergence only

(paternal) — direct, authoritative, declarative

(maternal) — warm, permissive, inviting

(pause Ns) — silent beat ~N seconds

(WATCH) — eyes on the client's eyelids/blink rate/breath

(WAIT-blink) — hold the line until you SEE the next blink, then resume on it

(TOUCH) — light contact on the forehead at the instant the eyes close

(PUSH) — gently lift-and-drop the client's hands off the lap to prove limpness

(SNAP) — finger snap immediately before "Deep sleep"

(breath-match) — sync your own out-breath and cadence to the client's breathing

1. When to use it (selection criteria)

Primary tell (from interview): during the intake conversation the client's eyes repeatedly fade, glaze, or blink heavily while listening — they are already offering eye-fatigue. Eye Fascination meets them where they are.

Good fit: clients who fixate/zone out easily; high visual suggestibility; anyone who found arm-raising awkward; a fast, low-touch secondary induction in subsequent sessions.

Poor fit: dry-eye / contact-lens discomfort, recent eye surgery, photophobia, uncontrolled watering; anyone who reports staring makes them anxious. (Switch to finger-spread or arm-raising.)

Role in the session: most often a **secondary induction / deepener** (client already knows "deep sleep"); can serve as a primary induction when the eye-fade tell is strong.

Prep: glasses off, contacts considered, gum out, phone silenced; a fixation point slightly **above** eye level so the upper eyelids are already working against gravity (ceiling spot, a raised pen, or your raised finger).

2. Fixation setup (shared)

"From where you're sitting, pick a spot on the ceiling a little above your eye line — so the eyes have to look up just slightly to find it — and rest your gaze there." (calm, unhurried)

"Let the eyes settle on that one point... keep them there, without moving them." (breath-match — take a few seconds here and let your out-breath fall into their rhythm) (WATCH the blink rate — this is your baseline) Note which eye tires first and whether blinks are already frequent; that read tells you how fast the lane below can move.

3A. Permissive lane (Emotional / inferred — workbook baseline)

(maternal, slow, spacious; lots of pauses; let their own imagination do the closing)

Awareness of the eye muscles: "as you keep looking at that spot — the gaze may drift and come back, gently, easily — you can start to notice the little muscles around the eyes... how they hold the eyes up... and the eyelids... what they feel like." (pause 2s)

First blink acknowledgment: "you may (WAIT-blink) begin to blink a bit more often (WAIT-blink) — that's it — and the breathing might deepen as you relax."

Heaviness offered, closure withheld: "as the eyes rest on that spot, the mind wrapped around it, the eyelids may begin to feel heavy... the eyes may want to close. But not just yet — let them stay open a little longer."

(holding closure back builds the urge — this is the engine of the technique)

Blink escalation: "and blinking (WAIT-blink) — that's it — even more now. You might notice that blink is a little slower than normal..." (voice ↓)

Blur/waver imagery (visual convincer): "many people, entering hypnosis, find the focus going soft... the vision blurring... and as you look at that spot it might waver... or blur... or seem to shift shape or color." (pause 2s)

"That's it."

Limb heaviness link: "you may notice the arms and legs growing heavy as the eyelids grow heavier and heavier... and how good it would feel to let those tired eyes just relax and close."

Permission to close: "as you focus on that spot and the eyes grow more tired, heavier, more fatigued... when you're ready, you can let them close." (WATCH — if already closed, skip straight to §4) (pause)

Reward the closure: "and closing the eyes means you can relax now... it feels good... peaceful... as you really let yourself go." → continue to §4.

3B. Paternal-Rapid lane (Physical / literal — the classic Kappas variant)

(paternal, RAPID, continuous — no gaps for the critical mind; message-unit overload drives the lids down; this is the lane in the practitioner brief)

Immediately load, direct and fast: "Keep staring at that spot — don't take the eyes off it — and as you stare, the eyelids are already starting to feel heavy... heavy and tired... the muscles around the eyes working, tiring, wanting to rest." (rapid, no pause) (WATCH the lids)

Stack cause→effect continuously: "the longer you stare, the heavier they get — heavier and heavier — the eyes are watering, tiring, blinking (WAIT-blink) — that's it — and every blink makes them heavier, and the heavier they get the more they blink, and the more they blink the heavier they get." (loop the reciprocal chain fast — blink feeds heaviness feeds blink)

Drive toward closure (unlike the permissive lane, here you push the lids DOWN): "so heavy now they want to close — let them get heavier — heavier — so heavy they're going to close, closing down now, heavier and

heavier, closing... closing..." (rapid, insistent, voice ↓ driving)

Closure + forehead touch (the convincer): the instant the eyelids shut — (TOUCH: light contact to the center of the forehead) — (SNAP) "Deep sleep." (voice ↓↓, strong) (the touch localizes attention and marks the moment; timing is everything — TOUCH on the close, not before)

Limpness proof (PUSH): immediately lift the client's hands a few inches off the lap and let them drop — "and the whole body going loose... limp... heavy... every muscle letting go, deeper with each breath." (PUSH proves catalepsy/release to both of you; if the hands hold tension, repeat the drop and deepen the language before moving on) (WATCH — hands should fall dead-weight)

Lock it: "that's it — loose and limp all over, deeply relaxed." → continue to §4.

4. Physiology confirm + double-breath drop (shared)

Name the body changes as convincers (WATCH and time to what's actually happening): breathing deepening, limbs heavy, a settling in the chest. (permissive: offered softly; paternal: stated as fact)

Double-breath deepener: "breathe in for a moment... and breathe out — and let that relaxation double through the whole body." (pause on the exhale)

"Deeper and deeper... all the way down..." (voice ↓)

5. Count 5 → 0 to Deep Sleep (shared)

"5 — the body relaxing... 4 — the mind letting go... 3 — emotionally calm... 2... 1... 0 —" (SNAP) "Deep sleep." (each number voice ↓ a step; permissive slow, paternal brisk)

6. Post-hypnotic suggestion (REQUIRED — full anatomy, first session and after each deepener)

cue words ("deep sleep") + purpose ("for the purpose of hypnosis") + consent ("and with your permission") + speed/quality triple ("quickly, calmly, and deeply") + somatic tag ("and the physical body relaxes").
(measured, formal)

→ From here proceed into deepeners / progressive relaxation / suggestions as the session plan dictates, or count out (0→5, eyes open wide awake) if closing.

Appendix A — Lane selector

Read from interview	Lane	Tonality	Closure move	Convincer
Emotional / inferred; needs control & safety	3A Permissive	maternal, slow	invite ("when you're ready")	blur/waver imagery
Physical / literal; needs to feel it	3B Paternal-Rapid	paternal, rapid	drive lids down	forehead touch + hands-drop limpness
Blended (40-60%)	open 3A, finish 3B	warm→direct	invite, then drive if lids linger	imagery then touch

Appendix B — Touch & consent (paternal lane)

Forehead contact and lifting the hands are **physical touch** — get explicit permission in the pre-talk ("I may lightly touch your forehead and lift your hands to help you relax — is that alright?") and honor a no by running the permissive lane instead. (with-permission is part of the PHS contract and a scope/boundary requirement)
Touch is light and brief; the hand-drop is a gentle lift-and-release, never a yank. Watch for any flinch and stop.

Appendix C — Age / condition swaps

Band / condition	Adjustment
Child (<13)	permissive lane only; playful ("let those sleepy eyes get heavy like at bedtime"); no forehead touch unless guardian present and consenting; short
Teen	permissive or gentle paternal; standard
Adult	either lane per read
Elder	permissive lane; go slow; the hands-drop limpness proof is fine, skip anything startling
Dry eye / contacts / photophobia	do not use — switch to finger-spread or arm-raising
Anxious about eye contact/staring	do not use — switch induction

Appendix D — Fault table

What you see	Likely cause	Fix
Eyes won't close (permissive)	closure invited too early, urge not built	keep withholding ("not yet"), stack more heaviness + blur, let blinks accumulate before permitting
Eyes won't close (paternal)	pace too slow → critical mind re-engaged	speed up, remove pauses, tighten the blink↔heavy reciprocal loop
Client tears up / eyes water hard	normal at first, or dryness	if comfortable, reframe as relaxation and let them close; if irritation, allow the close early and move to §4
Lids close but hands stay tense (paternal)	not yet released	repeat the hand-drop, deepen limp language, add a breath-drop before counting
Client closes eyes then re-opens	startled by touch or snap	soften next touch, drop voice, reassure ("that's fine, just let them close again")
Fixation point too low	eyelids not fatiguing	raise the spot above eye level so gravity works the upper lids